Johnson&Johnson

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**RE:** J&J Response to Department of Health and Human Services (HHS) Request for Information on Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.

Dear Secretary of the Department of Health and Human Services Robert F. Kennedy Jr.,

Thank you for the opportunity to provide comments on the Adoption and Use of Artificial Intelligence (AI) as Part of Clinical Care. We commend the Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC) actions to consider input from varied stakeholders on how to accelerate the uptake of AI technology in healthcare, foster public trust and confidence in modern technology solutions, to reduce uncertainty that impedes AI innovation, and improve health outcomes for patients, caregivers, and communities.

At **Johnson & Johnson**, our employees are driven by a passion to achieve the best version of health for everyone, everywhere, for as long as possible. Innovating across the full spectrum of healthcare solutions puts us in a unique leadership position today to deliver the breakthroughs of tomorrow. Our strength in both biology and medical technology means we are accelerating advances in care – from cell therapy to AI-assisted robotic surgery. We are using our wide range of expertise to address healthcare challenges that can be tackled by both medical technology and innovative medicine such as cancer, cardiovascular disease, and eye health.

## How We are Using AI

At J&J, we are applying AI across our business, focusing on finding solutions to big challenges and advancing our impact on patients. We believe AI, when used at scale, has the potential to accelerate our ability to advance human health. AI allows us to analyze larger-than-ever data sets in ways never before possible to find patterns and trends that are helping to transform our innovative engine. We are using AI, leveraging de-identified data, in a multitude of ways across our sectors:

> *Accelerating drug discovery:* We are leveraging state-of-the-art AI/machine learning (ML) technologies across drug discovery; from identifying novel targets that we can modulate to combat disease, to enabling the identification of molecular starting points for those targets, to optimizing those molecules to design and develop a drug candidate.
>
> *Speeding enrollment and research in clinical trials:* We are using ML algorithms to analyze real world health data from sources like de-identified electronic medical records, labs, claims and imaging to broaden recruitment and identify and prioritize geographies and healthcare organizations where more patients are being treated.
>
> *Driving precision medicine:* We are applying to AI and ML to data sets – from digital images to sound recordings – to identify novel biomarkers and clinical endpoints that can help us develop and deliver the right medicines for the right patients at the right time.

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*Harnessing AI for electro-anatomical mapping of the heart:* During catheter ablation procedures for atrial fibrillation, it is critical for electrophysiologists to “see” inside the heart. Our CARTO™ 3 System – the leading 3D heart mapping system – features AI to reconstruct the left atrial anatomy of the heart. By eliminating the need for manual contouring, the system represents a new approach to anatomy creation, improving the efficiency of the procedure workflow.

*Simplifying operative experiences for physicians:* Our AI-powered VIRTUGUIDE™ software automates patient analysis and correction to treat bony deformities of the foot and ankle. By using AI to analyze the patient’s anatomy, this software outputs the instrument needed to treat the specific patient and recommends a correction plan, which allows for intra-operative simplicity, fewer instruments, and reduced surgery time.

*Accelerating surgeon and care team training:* Our first-of-its-kind laparoscopic surgical simulation platform integrates augmented reality, AI-guided assessment, and tactile feedback, to help enhance the surgical skills and training experiences of suturing for general, gynecological, and laparoscopic surgeons.

AI-based technologies are revolutionizing healthcare and are already changing how diseases are understood, diagnosed, and treated; how healthcare systems and healthcare professionals (HCPs) deliver care with enhanced insights and improved workflows; and how new healthcare solutions are developed. Safely and responsibly harnessing AI’s potential in healthcare requires the implementation of a well understood, agile and future-proof policy framework that encourages innovation and research partnerships, builds a data culture that respects personal data privacy and security, supports education and upskilling in the healthcare and regulatory workforce and encourages global cooperation and alignment, where appropriate.

## Barriers to Adoption, Trust, and Governance of AI in Clinical Care

Patients and caregivers hope that AI can improve access to timely and cost-effective care while still offering personalized treatments without care fragmentation. On the other hand, they are concerned with data privacy, inaccurate information/medical recommendations, and loss of human connection. Patients want to know that their healthcare providers are still at the helm of decisions being made.

The nuanced, context-dependent nature of clinical medicine creates barriers to AI implementation. While AI can provide directional information, medical decision making requires integrated and complex information. Medical decisions must account for highly variable patient presentations, comorbidities, social determinants, and evolving evidence, making it difficult to establish trust that AI algorithms have the appropriate training and validation. AI systems depend on patterns of information that do not account for nuanced clinical information. There is danger in blindly adopting AI output without contextual considerations by the clinician.

Clinical data used to develop AI models are typically pulled from published studies and guidelines. However, the quality of studies can vary widely, and AI models often cannot

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discern between well conducted and validated information and may pull from poorly conducted studies which reduces model reliability. Often, data sources are not readily disclosed, and some AI models will hallucinate sources. This creates mistrust and presents as a barrier to medical professionals using AI.

As data fuels AI, we need to access large enough data sets to train models. This requires rational approaches to determining data ownership, control, and governance structures. We must also consider the quality of the data sets: if data used to train the AI model has limitations and does not adequately represent the target population, one could have outputs that are not representative enough.

**Risk-based and clear and predictable regulatory framework**

We understand that one important barrier to private sector innovation in AI for healthcare and its adoption and use in clinical care is regulatory uncertainty. Evolving and inconsistent requirements for AI in healthcare surrounding validation standards and explainability can create unclear approval pathways and investment risk, slowing development and deployment. Greater clarity and alignment across regulatory frameworks can help unlock innovation and support responsible scaling of AI in healthcare.

Traditional AI has been incorporated into medical devices for three decades. AI-driven medical devices are already highly regulated both in pre and post market and the existing regulatory requirements are appropriate for these devices. Any use of third-party certification will lead to redundant activities and documentation and may lead to disclosure of commercial confidential information and inappropriate release of private healthcare data.

Consideration should be given to identifying if there are any specific updates to the FDA regulatory processes that would be appropriate to apply to generative or agentic AI. These evidence standards should be specific to the nature of the use case for AI and not the model itself.

The CMS HTI-1[1] rule has been interpreted to apply to products regulated as medical devices if its training data are Electronic Health Records. Clarification on the scope of the rule would ensure that medical device manufacturers understand all the requirements that apply to their AI-driven medical devices.

We would welcome AI evaluation methods which ultimately lead to AI being deployed in ways which are helpful for healthcare professionals and beneficial to the patients they serve. These AI evaluation methods may include pre- and post-deployment testing and other robustness measures, as they may clarify how a non-medical device AI may perform in practice, an important consideration for healthcare providers who may be

[1] https://healthit.gov/regulations/hti-rules/hti-1-final-rule/

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hesitant to use AI otherwise. One way HHS could support these processes would be through incentives, such as grants or prize competitions.

AI in clinical care may lead to an increased level of sophistication for privacy and security programs, and the intersection of AI and liability can be unclear. There is a lack of clarity on who bears responsibility when AI tools indirectly cause harm. Indirect harm may be caused by algorithmic errors, bias-related harms, or failures in continuous learning of the AI system. HHS should help address not only reducing the probability of harm through guidance but also providing direction on who bears responsibility.

**Safeguarding Data Privacy**

Another key element for safe, ethical, and transparent AI use is privacy. We operate in a highly regulated industry where we use a variety of types of data, including administrative and claims data, clinical data, genomic data, patient-generated data, surgical video, and social determinants of health data that move through a variety of transmission networks. This data is essential to continued innovation, discovery, evaluation, and speed for the delivery of healthcare products and services to patients and consumers.

We support adoption of a comprehensive national privacy law and associated standards to help ensure a more consumer- and patient- friendly approach to managing personal information while ensuring consistent privacy protection, allowing a greater flow of data, and maintaining adequate protections. A federal law would help reduce variability across multiple governments and government agencies, consequently reducing regulatory costs and increasing competitiveness to attract investments.

**Enhanced Interoperability and Data Standards**

We would welcome promoting and incentivizing enhanced interoperability across electronic health records (EHR) through the use of common data models and standards such as Fast Healthcare Interoperability Resources (FHIR). Promoting the adoption of common data models enables larger, more diverse data sets for model training and validation of AI. In turn, this enablement can accelerate AI development across the healthcare ecosystem, ultimately driving more efficient and better healthcare outcomes, while expanding the global market reach of U.S. healthcare and technology companies.

Global data standards and interoperability of data and electronic health records are key to fuel research and innovation, thus enhancing patient outcomes. Global standards for data quality would contribute to the dual beneficial outcome of the enhancement of AI models through access to more data for training purposes, and the interoperability of electronic health record systems increasing data availability for R&D and improving patient outcomes, such as disease predictability and personalized treatment. This will broaden patients' access to treatment and further enhance AI utilized in clinical care, increasing market opportunities and patient outcomes.

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**Research Priorities to Accelerate Responsible AI Adoption**

We believe HHS should work to prioritize and fund research on the impact of AI on patient care, patient safety, and clinician knowledge. It is unknown how the potential for dependency on AI for medical students and other healthcare providers in training will impact the competency of those individuals. Research is needed to better understand new conjecture to promote adoption or potential guardrails for use of AI in academic and clinical practice.

We would welcome HHS prioritizing AI research that delivers clear, clinically meaningful explanations at the point of care, paired alongside continuous real-world performance monitoring to verify AI remains consistent in the quality of its performance after deployment. Together, these capabilities can strengthen clinician trust and accelerate adoption of AI into routine clinical practice.

Additionally, HHS should actively catalyze research on privacy-enhancing technologies (PETs), including federated learning, secure multi-party computation, encryption, and high-quality synthetic data, by funding targeted grants, CRADAs (cooperative research and development agreements), and prize competitions, creating de-identified benchmark datasets and interoperable standards, and developing clear evaluation frameworks and metrics that quantify utility/privacy tradeoffs. By sponsoring pilot implementations in real-world health systems, issuing guidance on acceptable risk thresholds and documentation expectations, and supporting workforce training to operationalize PETs in clinical workflows, HHS can both lower adoption barriers and ensure that research outputs are reproducible, interoperable, and aligned with patient privacy, public trust, and continued innovation.

**Modernizing Payment Systems and Market Access for AI in Clinical Care**

Ensuring payor coverage of AI-powered and enabled health technologies is a challenge given the complexity of existing healthcare payment systems, the divergence of intellectual property policies, and the lack of standardized, trustworthy, and transparent testing of AI models. To ensure smooth adoption and coverage of new technologies in various healthcare systems across markets, we must first ensure the quality, accuracy, reliability, transparency, and representativeness of data for AI models. It is necessary to establish - where they do not already exist - clear evidence and data requirements for lifecycle management of AI-enabled health technologies.

Where appropriate, create public and private sandboxes for developers and deployers to test and certify their solutions in a controlled environment, such as on AI model performance, health outcomes, data accuracy, and representativeness in AI models.

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We recognize the powerful promise of AI in healthcare, and we know this timely effort requires collaboration and partnership. We value our role as an innovator in healthcare, contributing new ideas, solutions, technology, partnerships, and perspectives on AI policy.

J&J looks forward to serving as a resource and providing additional thoughts and contributing to efforts that ensure AI is ethically, safely, and efficiently researched, developed and integrated into our healthcare system and our society. If you have any questions or if we can provide any assistance on issues related to healthcare, please e-mail Carla Cartwright at ccartwr@its.jnj.com.

Sincerely,

Carla Cartwright
Vice President, Global Digital &
Regulatory Policy

Carla Cartwright